

Eczema and Dermatitis

Patient Group Direction, version 002, issued 7 September 2026. Two arms: clobetasone butyrate 0.05% and betamethasone valerate 0.1%

Scope of this PGD

This PGD authorises the supply of a topical corticosteroid for mild to moderate eczema or dermatitis in patients aged 12 years and over. Potency is matched to severity: clobetasone butyrate 0.05% for mild disease, betamethasone valerate 0.1% for moderate disease.

- **Betamethasone valerate 0.1% is not authorised on the face, eyelids, flexures or genital skin. Use clobetasone on those sites, or refer.**
- Maximum four weeks of continuous daily treatment in either arm, and a maximum of three courses in any twelve months before GP review.
- Quantity is by treated body area. See Appendix 1.

Concurrent supply for infected eczema

Where eczema is complicated by MILD, LOCALISED secondary bacterial infection, a patient may receive a topical corticosteroid under this PGD and an oral antibiotic under the Skin and Soft Tissue Infection PGD at the same consultation. Both supplies are recorded in one consultation record. Where the infection is not mild and localised, or any red flag from the infection PGD is present, refer and supply neither.

Arm 1 — Clobetasone butyrate 0.05% (moderate potency)

Patient Group Direction for the supply of clobetasone butyrate 0.05% cream or ointment for MILD eczema or dermatitis in patients aged 12 years and over.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. Neither product is a controlled drug, so registered pharmacy technicians may lawfully supply under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- Must have completed training relevant to this condition, documented and overseen by the Get Real Health team.
- Must be able to distinguish eczema and dermatitis from the conditions excluded below, in particular rosacea, perioral dermatitis, acne and eczema herpeticum.
- Must be able to apply the fingertip unit table in Appendix 1 and to judge the treated body area.
- Must previously have used PGDs to supply medication.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.

The PGD

Indication	Short-term treatment of mild eczema or dermatitis in patients aged 12 years and over where a topical corticosteroid is appropriate. This is the first-line potency under this PGD.
Inclusion criteria	• Aged 12 years and over. • Mild eczema or dermatitis: limited erythema and scaling, little or no excoriation, not markedly affecting sleep or daily activity. • Diagnosis clear and consistent with eczema or dermatitis. • Fewer than three courses supplied under this PGD in the preceding 12 months. • Valid informed consent given. • No exclusion criterion present.
Exclusion criteria	Refer, do not supply, where any of the following is present. • Suspected eczema herpeticum: rapidly worsening, painful, punched-out or clustered vesicular lesions, or the patient is systemically unwell. Arrange same-day assessment. Topical corticosteroids are the classic aggravator and this is the one dermatological emergency this service will meet. • Signs of secondary bacterial infection, unless being treated concurrently under the Skin and Soft Tissue Infection PGD (see the note on concurrent supply below). • Rosacea, perioral dermatitis or acne. All three are worsened by topical corticosteroids and all three realistically present as "a rash". • Use on broken or ulcerated skin. • Hypersensitivity to clobetasone or to any component of the preparation. • Pregnancy or breastfeeding. • Under 12 years of age. • Application to the eyelids. Clobetasone may be used on the face, flexures and genital skin for up to 7 days only. • A third course already supplied in the preceding 12 months. Refer to the GP. • Diagnosis uncertain, or the rash does not fit eczema or dermatitis.
Cautions	• Use the lowest potency likely to control the flare, for the shortest time. Step up only on inadequate response and step down once control is achieved.

	- Do not use under occlusion. - For external use only. Keep out of the reach of children. In case of accidental ingestion seek professional assistance. - Advise on appropriate emollient use alongside the corticosteroid, following the sequence set out in the patient information. - Therapy should be discontinued gradually once control is achieved, with an emollient continued as maintenance.
Actions if excluded or declines	Explain why treatment cannot be supplied. Refer to the GP, and arrange same-day assessment where eczema herpeticum or spreading infection is suspected. Advise on emollient use, which remains appropriate in all cases. Document the advice and the decision.

The medicine

Name, form and strength	Clobetasone butyrate 0.05% w/w cream or ointment.
Legal category	POM.
Route and method	Topical. Apply a thin layer to the affected area only. Ointments suit dry, scaly skin; creams suit moist or intertriginous areas. Wash hands after application unless the hands are being treated.
Dose and frequency	Apply a thin layer once or twice daily to affected areas.
Quantity to be supplied	By treated body area, using the fingertip unit table in Appendix 1. Limited area (up to two adult palms): 15g. More extensive involvement: 30g. Maximum treated area under this PGD: 10% of body surface, roughly ten adult palms. Above that, refer.
Maximum treatment period	Up to 7 days initially, then review. Maximum 4 weeks of continuous daily treatment. Longer than that requires GP review. Maximum three courses in any 12 months before GP review.
Adverse effects	Common: burning, stinging or itching on application. Uncommon: skin thinning, striae, telangiectasia, folliculitis, worsening of untreated infection. Rare: systemic corticosteroid effects with extensive or prolonged use.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	- That valid informed consent was given. - Patient name, address, date of birth, and the GP with whom they are registered. - Name and registration number of the healthcare professional supplying. - Which product and potency was supplied, and the reason for that choice. - The body sites treated and the approximate area, and the quantity supplied. - Batch number and expiry date. - How many courses the patient has had in the preceding 12 months. - Advice given, including advice given if excluded or declining treatment. - Details of any adverse drug reactions and the actions taken. - That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Electronic records may be made. Adults: 8 years. Under 18s: until the 25th birthday.

Storage	Store below 25C.
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Information for the patient

Written information	Supply the patient information leaflet for the product given, and the Get Real Health fingertip unit sheet.
Counselling	● Apply the steroid FIRST, in a thin layer, to the affected skin only. Wait for it to absorb, then apply your emollient. Do not apply emollient immediately over a freshly applied steroid. ● Use emollient often and generously, including when the skin looks clear. That is what keeps flares away. ● Use only the amount shown on the fingertip unit sheet. More is not better and increases the risk of skin thinning. ● Stop when the flare settles and continue with emollient alone. ● Seek advice the SAME DAY if the rash becomes rapidly worse, painful, blistered or punched out in appearance, or if you feel unwell with it. ● Seek advice if the rash weeps, crusts or becomes yellow, which may mean infection. ● Come back if it has not improved within 7 days.
Follow-up	Review at 7 days if not improving. Do not continue daily treatment beyond 4 weeks without GP review. If your eczema returns within three months of finishing a course, see your GP rather than returning for another supply.

Arm 2 — Betamethasone valerate 0.1% (potent)

Patient Group Direction for the supply of betamethasone valerate 0.1% cream or ointment for MODERATE eczema or dermatitis in patients aged 12 years and over.

This arm is restricted to moderate disease and is not authorised on the face, eyelids, flexures or genital skin. Where those sites are involved, use clobetasone or refer.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC. Neither product is a controlled drug, so registered pharmacy technicians may lawfully supply under this PGD (Human Medicines Regulations 2012 as amended, 26 June 2024).
- Must have completed training relevant to this condition, documented and overseen by the Get Real Health team.
- Must be able to distinguish eczema and dermatitis from the conditions excluded below, in particular rosacea, perioral dermatitis, acne and eczema herpeticum.
- Must be able to apply the fingertip unit table in Appendix 1 and to judge the treated body area.
- Must previously have used PGDs to supply medication.
- Must work in compliance with the SOPs of their own employer and practise only within the bounds of their own competence.

The PGD

Indication	Short-term treatment of moderate eczema or dermatitis in patients aged 12 years and over, on body sites other than the face, eyelids, flexures and genital skin, where a moderate-potency corticosteroid has been insufficient or the severity warrants a potent product from the outset.
Inclusion criteria	• Aged 12 years and over. • MODERATE eczema or dermatitis: marked erythema, excoriation or lichenification, or disease disturbing sleep or daily activity. • Affected sites do NOT include the face, eyelids, flexures or genital skin. • Diagnosis clear and consistent with eczema or dermatitis. • Fewer than three courses supplied under this PGD in the preceding 12 months. • Valid informed consent given. • No exclusion criterion present.
Exclusion criteria	Refer, do not supply, where any of the following is present. • Suspected eczema herpeticum: rapidly worsening, painful, punched-out or clustered vesicular lesions, or the patient is systemically unwell. Arrange same-day assessment. Topical corticosteroids are the classic aggravator and this is the one dermatological emergency this service will meet. • Signs of secondary bacterial infection, unless being treated concurrently under the Skin and Soft Tissue Infection PGD (see the note on concurrent supply below). • Rosacea, perioral dermatitis or acne. All three are worsened by topical corticosteroids and all three realistically present as "a rash". • Use on broken or ulcerated skin. • Hypersensitivity to betamethasone or to any component of the preparation. • Pregnancy or breastfeeding. • Under 12 years of age. • Application to the face, eyelids, flexures or genital skin. Betamethasone valerate 0.1% is not authorised on these sites under this PGD; use clobetasone or refer. • A third course already supplied in the preceding 12 months. Refer to

	the GP. • Diagnosis uncertain, or the rash does not fit eczema or dermatitis.
Cautions	• Use the lowest potency likely to control the flare, for the shortest time. Step up only on inadequate response and step down once control is achieved. • Do not use under occlusion. • For external use only. Keep out of the reach of children. In case of accidental ingestion seek professional assistance. • Advise on appropriate emollient use alongside the corticosteroid, following the sequence set out in the patient information. • Therapy should be discontinued gradually once control is achieved, with an emollient continued as maintenance. • This is a potent corticosteroid. The risk of skin atrophy, striae and telangiectasia is materially higher than with clobetasone, and is greatest at thin-skinned sites, which is why those sites are excluded.
Actions if excluded or declines	As Arm 1. Where the patient has moderate disease on an excluded site, supply clobetasone under Arm 1 if appropriate, or refer.

The medicine

Name, form and strength	Betamethasone valerate 0.1% w/w cream or ointment.
Legal category	POM.
Route and method	Topical. Apply a thin layer to the affected area only, avoiding the face, eyelids, flexures and genital skin. Wash hands after application unless the hands are being treated.
Dose and frequency	Apply a thin layer once or twice daily to affected areas.
Quantity to be supplied	By treated body area, using the fingertip unit table in Appendix 1. Limited area (up to two adult palms): 15g. More extensive involvement: 30g. Maximum treated area under this PGD: 10% of body surface. Above that, refer.
Maximum treatment period	Up to 7 days initially, then review. Maximum 4 weeks of continuous daily treatment. Maximum three courses in any 12 months before GP review.
Adverse effects	As Arm 1, with a higher incidence of skin atrophy, striae, telangiectasia and hypopigmentation. Systemic absorption is greater from large areas, thin skin and under occlusion.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given. • Patient name, address, date of birth, and the GP with whom they are registered. • Name and registration number of the healthcare professional supplying. • Which product and potency was supplied, and the reason for that choice. • The body sites treated and the approximate area, and the quantity supplied. • Batch number and expiry date.

	• How many courses the patient has had in the preceding 12 months. • Advice given, including advice given if excluded or declining treatment. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Electronic records may be made. Adults: 8 years. Under 18s: until the 25th birthday.
Storage	Store below 25C.

Information for the patient

Written information	Supply the patient information leaflet for the product given, and the Get Real Health fingertip unit sheet.
Counselling	• This is a stronger steroid. Do not use it on your face, eyelids, skin folds or genitals. • Apply the steroid FIRST, thinly, to affected skin only. Wait for it to absorb, then apply your emollient. • Use only the amount shown on the fingertip unit sheet. • Stop when the flare settles and continue with emollient alone. Do not use it continuously for more than four weeks. • Seek advice the SAME DAY if the rash becomes rapidly worse, painful, blistered or punched out, or if you feel unwell with it. • Come back if it has not improved within 7 days.
Follow-up	As Arm 1.

Appendix 1 — Fingertip units and treated area

One fingertip unit (FTU) is the amount squeezed from a standard nozzle along an adult index finger from the tip to the first crease. One FTU is approximately 0.5g and covers approximately two adult palms.

Adult, per application	• Face and neck: 2.5 FTU (clobetasone only). • One arm: 3 FTU. One hand, both sides: 1 FTU. • One leg: 6 FTU. One foot: 2 FTU. • Trunk, front: 7 FTU. Trunk, back including buttocks: 7 FTU.
Child 12 to 17, per application	• Use adult figures where the patient is of adult build; otherwise reduce proportionately and record the area treated.
Quantity guide	• Up to 2 adult palms treated: 15g is sufficient for a 7 day course. • More extensive: 30g. • Above 10% of body surface (about ten adult palms): refer rather than supply.

Authorisation

Version	v002
Supersedes	Version 001
Valid from date	7 September 2026
Expiry date	31 July 2027

Doctor	Name: Nitin Shori Job title: Medical Director, Get Real Health GMC: 6047293 Signed: N. Shori Date: 7 September 2026
Pharmacist	Name: Chris Pilkington Job title: Head Pharmacist, Get Real Health GPhC: 2046322 Signed: C. Pilkington Date: 7 September 2026

Both authorising signatories reviewed this version together on 7 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs.

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: Job title and organisation: Signature: Date:
Professional group signatory	Name:

	Job title and organisation: Signature: Date:
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Change history

Version	v002
Date	7 September 2026
Changes	• Full clinical review and reissue.